

العمر(تاريخ الميلاد) : 08-03-1945 المهنة : بدون عمل

التوقيع: (-----)

التعليق على التشخيص : -----

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / 2 ----- / 2 ----- / 2.1 ----- / 2.1

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: صباح الخير ابراهيم سيد عبدالله National ID: 24503082101201	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

L05723824



البنك الوطني
لبنان

صباح الخير
ابراهيم سيد عبدالله

قرية بونيت

مركز النبط - الجيزة

٢٤٥٠٣٠٨٢١٠١٢٠١



٢٠٢٦/٠١

٢٥٥٠٣٠٨٢١٠١٢٠١

بنون صل

ارضية

مسلمة

انثى

المطابقة مدارية حتى ٢٠٣٣/١/٠٩





Patient Name : الأستاذة / صباح الخير ابراهيم سيد
Patient No. : 24503082101201
Path No. : 435/24
Sex : Female
Age : 78 Y
Referring Doc. :
Request No. : 13003288
Request Date : 30/01/2024
Verification Date : 11/02/2024
Financial Category : أورام دار السلام - مبادرة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast lesion.

Nature of Specimen: Tru-cut biopsy, collected at 30/1/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using Ultraview HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against: ER clone (1E2), PR clone (SP1), Her2 clone (4B5) and Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 90% tumor cells (Allred score 8/8).
- Her2: Moderate complete membrane reaction in >10% tumor cells (score 2+) for SISH
- Ki67: 15% nuclear reaction of tumor cells.

Conclusion:

Left breast cancer, Tru-cut biopsy:
Invasive duct carcinoma, Grade II, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+) for SISH
- Ki67: Positive in 15% of tumor cells.

Signature

Verified by :

Dr. Heba Hasan

Printing Date : 13/02/2024 12:30:12

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Printed By : marwa mohamed yousse





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name: الأستاذة / صباح الخير ابراهيم سيد
Patient No.: 24503082101201
Path No.: 435/24
Sex: Female
Age: 78 Y
Referring Doc.:

Request No.: 13003288
Request Date: 30/01/2024
Verification Date: 11/02/2024
Financial Category: اورام دار السلام - مبادره
Department: Out Patient

Registered by: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Prof Dr Gamal Abdelatty
Dr. Duaa Mohamed

Verified by :
Dr. Heba Hasan

Printing Date : 13/02/2024 12:30:12

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt





Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name : الأستاذة / صباح الخير ابراهيم سيد
Patient No. : 24503082101201
Path No. : 435/24
Sex : Female
Age : 78 Y
Referring Doc :

Request No. : 13003288
Request Date : 30/01/2024
Verification Date : 03/02/2024
Financial Category : اورام دار السلام - ميله
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical data:
Left breast lesion.

Nature of specimen:
Tru cut biopsy, collected at 30/1/2024 , 1:55 pm

Gross examination
Multiple greyish white and yellowish tissue cores up to 1 cm , totally submitted.

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
* DCIS: Could not be detected within the limits of biopsy recieved.
* Angiolnvasion: Could not be detected within the limits of biopsy recieved.

Diagnosis:
Left breast lesion, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67)

Prof.Gamal Abdelatty
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

Printing Date : 13/02/2024 12:30:07

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt



Patient Name: Sabah Al Khair Ibrahim Sayed Abd Allah	Date : 30/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 80 Year(s)
Medical ID: 516/2024	Pat. ID: 46486

POST CONTRAST CT- EXAMINATION OF THE CHEST , ABDOMEN AND PELVIS

REVELED

****Known case of left breast cancer, as compared to the last study dated 6/2/2025**

Finding:

CHEST

- Right breast spiculated mass 2.9 x2 cm .
- Early interstitial pulmonary changes are seen involving both lungs manifested by subpleural lines with interstitial coarsening & fine reticular infiltrations associated with mild septal thickening.
- Few millimetric pulmonary nodules likely non specific.
- No significant pleural effusion, thickening or masses could be appreciated.
- The cardiac size is not enlarged with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Unremarkable non-enhanced appearance of the thoracic aorta and the great vessels.

ABDOMEN & PELVIS

- The liver is still seen enlarged with in-homogeneous CT texture . ***Rather regressive course*** of the previously noted hepatic focal lesions, only one can be depicted at segment VII which still measures about 3.7 x2.8 compared to last study 4x3cm mild intra hepatic biliary radical dilatation.
- ***Progressive course*** of Distal CBD rounded iso dense hypoenhancing lesion roughly about 2.8 x2.5 compared to last study 1.6x1.5 cm with still noted rather the same degree of cbd as well as , common hepatic ducts and intra-hepatic biliary radicals dilatation yet with pneumobilia.
- Contracted gall bladder ?? upon faintly dense stones.
- ***Still noted*** Bulky left adrenal gland with small nodular lesion measures 0.8x1.1 cm and 2HU (adenoma) similar smaller lesion is noted within the right adrenal gland.
- ***Rather stationary course*** regarding to size and number of Multiple enlarged celiac and porto-caval lymph nodes the largest is still measures about 1.5 X 3 cm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen. Few adjacent splenules noted.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Tiny left renal cysts are noted.
- Normal appearance of the pancreas, aorta, I.V.C.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

Still noted :

- Diffuse osteopenia.

- Grade I degenerative anterolateral disc lesions of L4 & L5 levels.

Opinion:

Reported case of left breast cancer shows

- **Regressive course of hepatic focal lesions**
- **Progressive course of biliary radicals dilatation down to distal CBD isodense lesion for ERCP correlation.**
- **Rather stationary course of previously noted hepatic focal lesion With contrast behavior suggestive of metastatic lesions**
- **Otherwise stationary course since the last study**

Dr. Reham Kfiedr, M.D.



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name: صباح الخليل إبراهيم عبد الله

Sex:

MDT form

3- Medical number:

Female

83187615

Old Med:

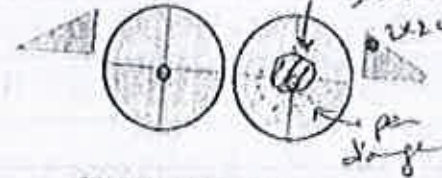
Date of MDT:

5/6/2024

2-ID number: 24503082101201

Patient Age: 79 Y

PS II
mass right
SC 3-4
2x2
per
change



Family history

Family history

Phone

Phone

01123076468

BMI

BMI

37

marital status

Marital status

HRS Score

HRS Score

3

GROUP

GROUP

A

Co morbidity

Co morbidity

HTN

Menopausal status

Menopausal status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT

Date of MDT

Date of MDT

20/03/2024

Breast Imaging:

LT multicentral LABC + Pathological

Clinical data

Mets Workup:

CT: Dialer IABR S
no focal lesion

BX

IDC

BX COMMENT:

GII with DCIS

ER

8/8

ER COMMENT:

PR

8/8

PR COMMENT:

Her 2 Neu

2

Her 2 Neu COMMENT:

SISH

SISH COMMENT:

Ki67

15%

T

4

N

3

M

0

Sonomammography

PATIENT HISTORY

clinically lt labc with nipple ulceration & central diffuse edema+pathologically enlarge axillary LN
T4N2M0

MDT OUTCOME & PLAN

FOR LEFT MRM (AI BRIDGING)

Normal

MDT OUTCOME & PLAN

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

Signature

SIGNATURE

SURGERY

SURGERY

Signature

RADIOLOGY

RADIOLOGY

Signature

Psychiatric

Psychiatric

Signature

PATHOLOGY

PATHOLOGY

Signature

Nutrition

Nutrition

Signature